

Postprandial angina pectoris: clinical and angiographic correlations.

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OBJECTIVES: This study was designed to determine the severity of coronary artery disease in patients with postprandial angina pectoris. BACKGROUND: Postprandial angina is a manifestation of coronary artery disease. Although seen in clinical practice, very little has been published about the syndrome, and no anatomic correlations have been described. METHODS: Questionnaires were given to 408 patients with chest pain and objective evidence of ischemia. Thirty-five patients (8.6%) were identified as having postprandial angina (Group A). The other 373 patients (Group B) had nonpostprandial angina and served as the control group. Coronary angiography was performed in all patients, and the results were analyzed. RESULTS: Postprandial angina was observed predominantly in men (91% vs. 66%, $p = 0.0036$). It was associated with a high incidence of rest angina (83% in Group A vs. 51% in Group B, $p = 0.0005$) and a very high incidence of left main (34% vs. 10%, $p = 0.0001$) and three-vessel (82% vs. 54%, $p = 0.001$) coronary artery disease. The ejection fraction was lower as well in these patients (0.39 vs. 0.47, $p = 0.046$). Postprandial angina occurred at rest and on exertion, most commonly after dinner. CONCLUSIONS: Postprandial angina is a likely marker of severe coronary artery disease and should be considered an indication for coronary angiography.